ISSUED, VALID FROM 30 SEPTEMBER 2026

Insomnia in Adults Aged 55 and Over (Circadin)

Patient Group Direction, version 010, issued 30 September 2026. Circadin 2mg prolonged-release melatonin tablets. Short-term treatment of primary insomnia, 55 years and over.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What primary insomnia means, and why it matters here

Primary insomnia is poor sleep that is not explained by another condition, another medicine, or a substance. Circadin is licensed for primary insomnia characterised by poor quality of sleep, and for nothing else.

- **Insomnia is very often secondary: to depression or anxiety, to pain, to obstructive sleep apnoea, to restless legs, to nocturia in prostatic disease, to alcohol, to caffeine, to shift work, or to a medicine the patient is already taking.**
- **Supplying melatonin to those patients does not treat anything. It delays the diagnosis, and in obstructive sleep apnoea and in depression that delay carries real risk.**
- **Take a proper history before supplying. If a cause is apparent, refer instead. That referral is worth more to the patient than the tablets.**

Circadin 2mg prolonged-release tablets

Summary of SmPC and NICE CKS guidance for prolonged-release melatonin (Circadin) in short-term insomnia in adults aged 55 and over

Circadin 2 mg prolonged-release tablets, Summary of Product Characteristics, Flynn Pharma Ltd for RAD Neurim Pharmaceuticals EEC SARL (PLGB 52348/0002), text revised 1 January 2021, emc updated 11 February 2025. NICE Clinical Knowledge Summary: Insomnia (Scenario: Managing insomnia and Prescribing information: Prolonged-release melatonin), last revised June 2026. Summarised 10 September 2026.

Licensed indication and age

The Circadin SmPC states that it is indicated as monotherapy for the short-term treatment of primary insomnia characterised by poor quality of sleep in patients who are aged 55 or over.

The SmPC explains the rationale as the role of melatonin in sleep and circadian rhythm regulation and the age-related decrease in endogenous melatonin production, so that melatonin may improve sleep quality particularly in patients over 55 with primary insomnia.

The safety and efficacy of Circadin in children aged 0 to 18 years has not been established.

NICE CKS says that for short-term insomnia (less than 3 months) where sleep hygiene has failed, daytime impairment is severe and insomnia is not likely to resolve soon, cognitive behavioural therapy for insomnia should be offered, with adjunctive short-term hypnotic treatment (a z-drug, or prolonged-release melatonin if over 55 years of age) considered.

NICE CKS says that for people over 55 with persistent insomnia, treatment with prolonged-release melatonin may be considered, with a recommended maximum duration of 13 weeks.

NICE CKS also states that hypnotics should not be prescribed routinely and should be used only for short courses if the person is acutely distressed, and that over-the-counter treatments for insomnia should not be recommended.

Dose, timing and duration

The recommended dose is 2 mg once daily, taken 1 to 2 hours before bedtime and after food.

The SmPC states that this dosage may be continued for up to thirteen weeks.

In the clinical trials described in the SmPC, patients received Circadin 2 mg every evening for 3 weeks, and a further trial showed the benefit observed after 3 weeks was maintained for up to 3 months.

Tablets should be swallowed whole to maintain the prolonged-release properties; crushing or chewing should not be used to facilitate swallowing.

NICE CKS gives the same dose: 2 mg once daily, 1 to 2 hours before bedtime and after food, for up to 13 weeks, for adults aged 55 years and over.

The SmPC notes that food delays absorption, with a later and lower peak plasma concentration in the fed state.

Contraindications and cautions

The only contraindication in the SmPC is hypersensitivity to melatonin or to any of the excipients.

The SmPC states that there is no experience of use in patients with liver impairment, that endogenous melatonin levels are markedly elevated in hepatic impairment because of decreased clearance, and that Circadin is therefore not recommended in patients with hepatic impairment.

The SmPC states that no clinical data exist on use in individuals with autoimmune diseases, and Circadin is therefore not recommended in patients with autoimmune diseases.

The effect of renal impairment on melatonin pharmacokinetics has not been studied and the SmPC advises caution in such patients.

Circadin contains lactose (80 mg per tablet); patients with rare hereditary problems of galactose intolerance, the LAPP lactase deficiency or glucose-galactose malabsorption should not take it.

The SmPC states that Circadin may cause drowsiness and should be used with caution if the effects of drowsiness are likely to be associated with a risk to safety.

The SmPC states that Circadin has moderate influence on the ability to drive and use machines.

NICE CKS says not to prescribe prolonged-release melatonin to people with autoimmune disease or hepatic impairment, and to prescribe with caution in renal impairment and in people with susceptibility to seizures because of a risk of increased seizure frequency.

NICE CKS advises telling the person not to drive if they feel sleepy, and that the DVLA must be informed if excessive sleepiness from any condition or medication is having, or is likely to have, an adverse effect on driving.

The SmPC states that use in pregnancy and in women intending to become pregnant is not recommended because of the lack of clinical data, and that breastfeeding is not recommended during treatment because exogenous melatonin is probably secreted into human milk.

Interactions

The SmPC states that melatonin metabolism is mainly mediated by CYP1A enzymes, so interactions arising from other substances' effects on CYP1A enzymes are possible.

Fluvoxamine increases melatonin levels (a 17-fold higher AUC and a 12-fold higher serum Cmax) by inhibiting its metabolism by CYP1A2 and CYP2C19; the SmPC says the combination should be avoided.

Caution should be exercised with 5- or 8-methoxypsoralen and with cimetidine, both of which increase melatonin levels by inhibiting its metabolism. Under this PGD, 5- and 8-methoxypsoralen are an exclusion (see the exclusion criteria); cimetidine is a caution.

Caution should be exercised in patients on oestrogens (for example contraceptive or hormone replacement therapy), which increase melatonin levels by inhibiting its metabolism by CYP1A1 and CYP1A2.
CYP1A2 inhibitors such as quinolones may increase melatonin exposure; CYP1A2 inducers such as carbamazepine and rifampicin may reduce plasma concentrations of melatonin.
Cigarette smoking may decrease melatonin levels because of induction of CYP1A2.
Alcohol should not be taken with Circadin because it reduces the effectiveness of Circadin on sleep.
Circadin may enhance the sedative properties of benzodiazepines and non-benzodiazepine hypnotics such as zaleplon, zolpidem and zopiclone; in a clinical trial co-dosing with zolpidem increased impairment of attention, memory and co-ordination compared with zolpidem alone.
Co-administration with imipramine increased feelings of tranquillity and difficulty performing tasks, and with thioridazine increased feelings of muzzy-headedness, although no clinically significant pharmacokinetic interactions were found.
NICE CKS lists the same interactions: alcohol, opioids and other CNS depressants (which can affect the ability to perform skilled tasks), fluvoxamine (avoid), quinolones and oestrogens (caution), carbamazepine and rifampicin (may reduce melatonin levels), and benzodiazepines and z-drugs (may enhance sedation and side effects).

Adverse effects

In clinical trials 48.8% of patients receiving Circadin reported an adverse reaction compared with 37.8% on placebo, but the rate of adverse reactions per 100 patient-weeks was higher for placebo than for Circadin.
The SmPC says the most common adverse reactions were headache, nasopharyngitis, back pain and arthralgia, which were common in both the Circadin and placebo groups.
Uncommon adverse reactions listed include irritability, nervousness, restlessness, insomnia, abnormal dreams, nightmares, anxiety, migraine, headache, lethargy, psychomotor hyperactivity, dizziness, somnolence, hypertension, abdominal pain, dyspepsia, mouth ulceration, dry mouth, nausea, hyperbilirubinaemia, dermatitis, night sweats, pruritus, rash, dry skin, pain in extremity, menopausal symptoms, asthenia, chest pain, glycosuria, proteinuria, abnormal liver function tests and weight gain.
Rare adverse reactions listed include herpes zoster, leukopenia, thrombocytopenia, mood alteration, aggression, agitation, depression, syncope, memory impairment, disturbance in attention, restless legs syndrome, poor quality sleep, visual disturbance, vertigo, angina pectoris, palpitations, hot flush, gastrointestinal upset, priapism, prostatitis, fatigue and thirst; hypersensitivity reactions, angioedema and galactorrhoea are listed with frequency not known.
In overdose, somnolence is the most reported event; the SmPC says drowsiness is to be expected, clearance is expected within 12 hours and no special treatment is required.
The SmPC states that when treatment stopped in trials, sleep variables gradually returned to baseline with no rebound, no increase in adverse reactions and no increase in withdrawal symptoms.

What to tell the patient

Take one 2 mg tablet once a day, 1 to 2 hours before bedtime and after food, swallowed whole and not crushed or chewed.
The SmPC limits treatment to a maximum of thirteen weeks, and NICE CKS gives the same 13-week maximum.
Do not drink alcohol with Circadin, because it reduces the effect on sleep.
Circadin may cause drowsiness; use caution with driving and other activities where drowsiness could be a safety risk, and do not drive if sleepy.
NICE CKS advises offering sleep hygiene advice, including a comfortable sleep environment, avoiding screens for at least an hour before bed, regular sleep schedules, avoiding daytime naps, avoiding caffeine after midday and nicotine, alcohol and large meals within 2 hours of bedtime, and avoiding vigorous exercise within an hour of bedtime.
NICE CKS recommends arranging follow-up in 2 to 4 weeks depending on the clinical situation, and reassessing for alternative diagnoses and the need for referral if symptoms have not improved.
NICE CKS says to refer to a sleep clinic or a specialist in sleep medicine or neurology if another sleep disorder such as a parasomnia, narcolepsy or obstructive sleep apnoea is suspected, or there is doubt about the diagnosis, and to seek specialist advice if primary care treatment has failed or the person is in an occupational at-risk group such as professional drivers.

What the sources do not say

The Circadin SmPC does not define short-term insomnia in weeks; the 3-week figure comes from the duration of its clinical trials rather than from a dosing instruction.
The Circadin SmPC does not state a caution about seizures; that caution appears only in NICE CKS.
NICE CKS does not give a separate list of contraindications for melatonin in the managing insomnia scenario beyond the general instruction not to prescribe hypnotics to older people or to women who are pregnant or breastfeeding; its melatonin-specific advice is in the prescribing information page.
Neither source gives advice on repeat courses after 13 weeks.

Patient Group Direction for the supply of Circadin 2mg prolonged-release melatonin tablets as monotherapy for the short-term treatment of primary insomnia characterised by poor quality of sleep, in patients aged 55 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for Circadin, the BNF and NICE CKS on insomnia.
- ALL USERS MUST BE ABLE TO DISTINGUISH PRIMARY FROM SECONDARY INSOMNIA and must take a history sufficient to do so, covering mood, pain, snoring and daytime sleepiness, alcohol, caffeine, shift work and the patient's full medicine list.
- All users must be able to screen for obstructive sleep apnoea and know that daytime sleepiness with snoring is a referral, not a supply.
- All users must be competent to deliver sleep hygiene advice as the first-line intervention, and to explain that it works better than the tablets over time.
- All users must know that Circadin is licensed from 55 years, and that this PGD does not authorise supply below that age on any basis.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must understand capacity and consent and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to this medicine.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Monotherapy for the short-term treatment of primary insomnia characterised by poor quality of sleep, in patients aged 55 years or over, in accordance with the current SPC for Circadin. Supply is within the marketing authorisation in every respect; this PGD does not authorise any off-label use.
Inclusion criteria	• **Aged 55 years or over. There is no upper age limit.** • Poor quality of sleep of at least 4 weeks' duration, affecting daytime functioning. • **A history has been taken and no secondary cause is apparent. See the exclusions.** • **Sleep hygiene advice given and, where the patient has not already tried it, a period of trying it agreed before or alongside supply.** • Not currently taking any hypnotic, sedative or other treatment for insomnia. • Valid informed consent obtained. • No exclusion criterion present.
Exclusion criteria: age and licence	**EXCLUDE anyone under 55 years. Refer to the GP.** • Circadin is licensed only from 55 years. Safety and efficacy in those aged 0 to 18 have not been established, and the licence does not extend to 18 to 54 either. • **Do not supply under 55 on an off-label basis under this PGD. If a**

	patient aged 18 to 54 needs melatonin, that is a prescriber decision. ● Tell the patient plainly why: the licence is based on the age-related fall in the body's own melatonin, which is why the evidence sits in the older group.
Exclusion criteria: secondary insomnia. Refer, do not supply	**Where any of these is present the insomnia is likely secondary. Treating it here masks the cause.** ● **Low mood, loss of interest, anxiety, or any current or suspected mental health condition. Early morning waking with low mood is depression until proved otherwise.** ● **Snoring with daytime sleepiness, witnessed apnoeas, or morning headache. Suspect obstructive sleep apnoea and refer. This is the exclusion most often missed and it carries cardiovascular and road-traffic risk.** ● Pain of any cause that wakes the patient or prevents sleep. ● An urge to move the legs at night, or leg discomfort relieved by movement. Suspect restless legs syndrome. ● Waking repeatedly to pass urine. Consider prostatic disease, diabetes or heart failure and refer. ● Shift work, or a sleep pattern driven by work or travel rather than by an inability to sleep. ● **Alcohol used to get to sleep, or any pattern of harmful drinking. Alcohol also reduces the effectiveness of Circadin.** ● Caffeine late in the day, or a high total daily intake, not yet addressed. ● **A medicine that could be causing the insomnia, for example a corticosteroid, a beta-agonist, an SSRI or SNRI, a stimulant, or a diuretic taken in the evening.**
Exclusion criteria: all other	**Refer, do not supply, where any of the following applies.** ● Known hypersensitivity to melatonin or to any excipient of Circadin. ● **Hepatic impairment of any degree. Circadin is not recommended by the SPC: the liver is the primary site of melatonin metabolism and endogenous levels are already markedly raised in hepatic impairment.** ● **Autoimmune disease. The SPC does not recommend Circadin, as no clinical data exist in this group.** ● **Pregnancy, planning pregnancy, or breastfeeding. Not recommended in any of these by the SPC.** ● **Taking FLUVOXAMINE. The combination is to be avoided: fluvoxamine raises melatonin exposure roughly seventeen-fold.** ● **Taking a benzodiazepine, a Z-drug (zopiclone, zolpidem, zaleplon), or any other hypnotic or sedative. Circadin enhances their effect, and co-dosing with zolpidem measurably worsened attention, memory and co-ordination.** ● Taking 5-methoxypsoralen or 8-methoxypsoralen. ● **Rare hereditary galactose intolerance, total lactase deficiency or glucose-galactose malabsorption. Circadin contains 80mg lactose per tablet.** ● Renal impairment where the pharmacist is not satisfied it is mild and stable. The effect of renal impairment on melatonin pharmacokinetics has not been studied. ● **A previous course of Circadin supplied under this PGD in the last 6 months, or 13 weeks of treatment already completed. Refer for review rather than continuing.** ● Insomnia present for less than 4 weeks. Give sleep hygiene advice and review; do not medicate a short-lived disturbance.

	• Valid informed consent not obtained.
Cautions	**DROWSINESS AND DRIVING. Circadin has a moderate influence on the ability to drive and use machines. Advise the patient not to drive or operate machinery if affected, and to be particularly careful the morning after the first few doses.** **Cimetidine, oestrogens including combined contraceptives and HRT, and quinolone antibiotics all raise melatonin levels by inhibiting its metabolism. Where the patient takes one of these, counsel on increased drowsiness. REFER INSTEAD OF SUPPLYING where the patient is also at risk of falls, or takes another sedating medicine (for example an opioid, a sedating antihistamine, a tricyclic antidepressant or a gabapentinoid). Otherwise supply may proceed with that counselling.** Carbamazepine and rifampicin lower melatonin levels and may make the treatment ineffective. Smoking has the same effect. **ALCOHOL SHOULD NOT BE TAKEN WITH CIRCADIN. It reduces its effectiveness on sleep, quite apart from its own effect on sleep architecture.** Take after food. Food delays absorption and lowers the peak level, which is what the licensed instruction is designed around; taking it on an empty stomach changes the profile. **Swallow whole. Crushing or chewing destroys the prolonged-release property and turns a night-long release into a single early peak.**
Actions if the patient is excluded or declines	Explain why supply is not appropriate and what to do instead. Where the exclusion is a possible secondary cause, name it and make the referral concrete: a patient who snores and is sleepy in the day needs a sleep apnoea assessment, and a patient waking early with low mood needs a mood assessment. Give the sleep hygiene advice in Appendix 1 regardless of whether anything is supplied, because it is the intervention with the better long-term evidence. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.

The medicine

Name, form and strength	Circadin 2mg prolonged-release tablets (melatonin). PLGB 52348/0002.
Legal category	POM.
Dose and frequency	2mg once daily, taken 1 to 2 hours before bedtime and after food. One tablet per day; the dose is not titrated under this PGD.
Quantity to be supplied	Up to 21 tablets per supply, being three weeks. A maximum of 13 weeks of treatment in total, and no further supply under this PGD within 6 months of completing a course.
Maximum treatment period	13 WEEKS. This is the licensed maximum duration and there is no extension under this PGD. A patient still not sleeping at 13 weeks needs review, not a repeat.
Route and method	Oral. Swallow whole with water. Do not crush, chew or halve.
Storage	Do not store above 25 degrees Celsius. Store in the original package to protect from light.
Drug interactions	Avoid: fluvoxamine, and any benzodiazepine or Z-drug. Exclusion: 5-methoxypsoralen and 8-methoxypsoralen (listed in the exclusion criteria; do not supply). Caution: cimetidine, oestrogens including combined contraceptives and HRT, and quinolones (refer instead where the patient is also at risk of falls or takes another sedating medicine; see the cautions).

	Reduced effect: carbamazepine, rifampicin, smoking. Alcohol should not be taken with Circadin. Melatonin induces CYP3A in vitro at supratherapeutic concentrations; clinical relevance unknown. Check the current SPC and BNF against the patient's full medicine list.
Adverse effects	Uncommon: headache, migraine, dizziness, somnolence, lethargy, psychomotor hyperactivity, irritability, nervousness, restlessness, insomnia, abnormal dreams, nightmares, anxiety, abdominal pain, dyspepsia, dry mouth, nausea, rash, pruritus, night sweats, hypertension, chest pain, asthenia, abnormal liver function tests, weight increase. Rare: altered mood, aggression, agitation, depressed mood, depression, syncope, memory impairment, visual disturbance, palpitations, angina, and haematological effects including leukopenia and thrombocytopenia. Not known: hypersensitivity reactions, angioedema. Refer to the current SPC.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • **AGE, confirmed as 55 or over.** • How long the insomnia has been present, and how it affects daytime functioning. • **THE SECONDARY-CAUSE HISTORY: mood, pain, snoring and daytime sleepiness, restless legs, nocturia, shift work, alcohol, caffeine, and the full medicine list, each asked and recorded.** • **That sleep hygiene advice was given, and what the patient had already tried.** • **Any previous Circadin supply, and the total weeks of treatment to date, from the pharmacy's own records and as reported by the patient (ask specifically about supplies made elsewhere).** • Name, form, strength, dose, quantity and date of supply. • **That the patient was advised about drowsiness and driving, and about alcohol.** • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Keep for 8 years.
Disposal	Advise the patient to return unused tablets to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product, together with the written sleep hygiene advice in Appendix 1.
Counselling	• **One tablet a day, 1 to 2 hours before you go to bed, after food.** • **Swallow it whole. Do not crush, chew or break it, or it will all be released at once instead of through the night.** • **It may make you drowsy. Do not drive or use machinery if you are affected, and take particular care the morning after the first few nights.**

	● **Do not drink alcohol with it. It makes the medicine work less well.** ● **It is not a sleeping tablet in the way you may be expecting. It works with your body clock and it works gradually. Expect an improvement in the quality of your sleep rather than being knocked out.** ● The sleep hygiene measures matter more than the tablet over time. Keep doing them. ● **This is a short course, up to 13 weeks. It is not a long-term treatment.**
Follow-up and when to seek advice	**See your GP rather than continuing if:** ● Your sleep has not improved after 3 weeks. ● **You feel low in mood, or you are waking very early and cannot get back to sleep.** ● **You are told you snore heavily or stop breathing in your sleep, or you are sleepy during the day.** ● **You develop a rash, swelling of the face, lips or tongue, or any reaction. Stop the tablets.** For a routine query about the medicine, contact the pharmacy.

Appendix 1: Sleep hygiene advice, to be given to every patient

Give this whether or not you supply. It has the better long-term evidence, and a patient who gets only a tablet has been short-changed.

Timing	• Get up at the same time every day, including at weekends. Fixing the wake time matters more than fixing the bedtime. • Only go to bed when sleepy. • If you are awake for more than about 20 minutes, get up, go to another room, do something quiet and dull, and go back when sleepy.
Daytime	• Get daylight in the morning, ideally outdoors. • Take regular exercise, but not in the 3 hours before bed. • Avoid daytime naps, or keep them under 20 minutes and before mid-afternoon.
Substances	• No caffeine after midday. That includes tea, cola and some painkillers, not just coffee. • Alcohol gets you to sleep and then wakes you at 3am. It is not a sleep aid. • Nicotine is a stimulant and also lowers melatonin levels.
The bedroom	• Cool, dark and quiet. • Bed is for sleep. No screens, no work, no television. • Turn the clock away from you. Watching the time makes it worse.
The evening	• Wind down for an hour before bed. Dim the lights. • Write tomorrow's worries down earlier in the evening, so they are not waiting for you at midnight. • Do not eat a large meal late.

Appendix 2: Key references

- Summary of Product Characteristics for Circadin 2mg prolonged-release tablets, current version. PLGB 52348/0002.
- NICE Clinical Knowledge Summary: Insomnia.
- British National Formulary.
- Human Medicines Regulations 2012, for the position on unlicensed medicines and PGDs.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

Authorisation

Version	v010
Supersedes	Version 009, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their	

signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

GET REAL HEALTH

Change history to version 003

Later versions are recorded in the Version and change record at the end of this document.

Version	v003
Date	8 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. ● The unlicensed melatonin arm is removed. Version 001 supplied melatonin 1mg, 3mg and 5mg tablets described in the document itself as unlicensed preparations. The Human Medicines Regulations 2012 do not permit an unlicensed medicine to be supplied under a PGD at all, so that arm had no lawful basis and could not be corrected. ● The age floor is 55, which is the licensed indication. Version 001 stated Circadin is indicated in patients aged 18 and over. It is licensed from 55, so every patient aged 18 to 54 was supplied off-label without being told. This version is written wholly within the licence rather than adding an off-label consent route. ● Maximum treatment period capped at 13 weeks, the licensed duration, with a maximum of 21 tablets per supply and no further course within 6 months. ● Secondary insomnia made an explicit set of exclusions with a required history: mood, pain, snoring and daytime sleepiness, restless legs, nocturia, shift work, alcohol, caffeine and the full medicine list. Obstructive sleep apnoea is called out as the one most often missed. ● Hepatic impairment, autoimmune disease, pregnancy, planned pregnancy and breastfeeding added as exclusions, all being groups the SPC does not recommend. ● Fluvoxamine, benzodiazepines and Z-drugs made exclusions rather than cautions. Fluvoxamine raises melatonin exposure roughly seventeen-fold, and co-dosing with zolpidem measurably worsened attention, memory and co-ordination. ● Lactose content flagged for hereditary galactose intolerance, total lactase deficiency and glucose-galactose malabsorption. ● Drowsiness and driving, and the interaction with alcohol, added to counselling and to the required records. ● Instruction to swallow whole and take after food, with the reason for each, since crushing defeats the prolonged release and food is what the licensed dosing is built around. ● Sleep hygiene advice added as Appendix 1, to be given whether or not anything is supplied.

Previous versions

001	Two arms: Circadin, stated as indicated from 18 years, and unlicensed melatonin 1mg, 3mg and 5mg tablets. Withdrawn 7 September 2026 on legal grounds.

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
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Version and change record
Version: v010
Issued: 30 September 2026
Supersedes: Version 009, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v009
Issued: 14 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision, joint sign-off 11 September 2026: 5- and 8-methoxypsoralen are an exclusion only; the drug interactions row no longer lists them under caution and states that they are an exclusion, the guidance summary notes that this PGD treats them as an exclusion, and the exclusion criterion is unchanged.
Joint sign-off by the Medical Director and the Head Pharmacist, 11 September 2026: the drafted texts of version 007 were reviewed together and confirmed, with the change above.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 56: for a patient taking cimetidine, an oestrogen or a quinolone, the cautions row and the drug interactions row now state that the patient is referred instead of supplied where they are also at risk of falls or take another sedating medicine; otherwise the caution stands and supply may proceed with counselling on increased drowsiness.
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Records row: states the sources for previous Circadin supply (pharmacy records and patient report, including supplies made elsewhere).

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v004

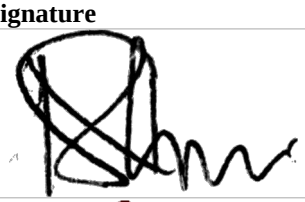
Issued: 10 September 2026

Supersedes: Version 003, 9 September 2026

Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.

Signed on behalf of Get Real Health

Version v010, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.